Effective Date: 04/01/2026
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Policy Number: C30614-A

Rinvoq (upadacitinib)

PRODUCTS AFFECTED

Rinvoq (upadacitinib)

COVERAGE POLICY

Coverage for services, procedures, medical devices, and drugs are dependent upon benefit eligibility as outlined in the member's specific benefit plan. This Coverage Guideline must be read in its entirety to determine coverage eligibility, if any. This Coverage Guideline provides information related to coverage determinations only and does not imply that a service or treatment is clinically appropriate or inappropriate. The provider and the member are responsible for all decisions regarding the appropriateness of care. Providers should provide Molina Healthcare complete medical rationale when requesting any exceptions to these guidelines.

DIAGNOSIS:
Rheumatoid Arthritis (RA), Psoriatic Arthritis (PsA), Atopic Dermatitis (AD), Ulcerative Colitis (UC), Ankylosing Spondylitis (AS), Non-Radiographic Axial Spondyloarthritis (nr-axSpA), Crohn's Disease (CD), Polyarticular Juvenile Idiopathic Arthritis (JIA), Giant Cell Arteritis (GCA)

REQUIRED MEDICAL INFORMATION:
This clinical policy is consistent with standards of medical practice current at the time that this clinical policy was approved. This clinical policy will be reviewed along with state and federal requirements, the benefit being administered, and formulary preferencing. If a drug within this policy receives an updated FDA label within the last 180 days, medical necessity for the member will be reviewed using the updated FDA label information. Coverage will be determined on a case-by-case basis.

A. FOR ALL INDICATIONS:
1. Member is not on concurrent treatment or will not be used in combination with another TNF-inhibitor, a biologic response modifier or other biologic DMARDs, Janus kinase Inhibitors, or Phosphodiesterase 4 inhibitor (i.e., apremilast, tofacitinib, baricitinib), or potent immunosuppressants such as azathioprine or cyclosporine, as verified by prescriber attestation, member medication fill history, or submitted documentation

B. RHEUMATOID ARTHRITIS (RA):
1. Documentation of moderate to severe rheumatoid arthritis diagnosis
AND
2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor

C. PSORIATIC ARTHRITIS (PsA):
1. Documentation of active psoriatic arthritis
AND

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2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor

D. ATOPIC DERMATITIS (AD):
1. Documented diagnosis of moderate to severe chronic atopic dermatitis (eczema)
   AND
2. Documentation of inadequate response, serious side effects, or contraindication to ONE of the following:
   a. topical corticosteroids
      OR
   b. preferred topical calcineurin inhibitor (tacrolimus, pimecrolimus)
      OR
   c. biologics (e.g., Dupixent, Dupixent, Adbry, Ebglyss, Cibinqo and Nemluvio)

E. ULCERATIVE COLITIS (UC):
1. Documentation of ulcerative colitis diagnosis with evidence of moderate to severe disease activity
   AND
2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor
   OR
3. If TNF-inhibitor is clinically inadvisable, documentation of treatment failure or serious side effects to a trial of a systemic therapy indicated for ulcerative colitis

F. ANKYLOSING SPONDYLITIS (AS):
1. Documented diagnosis of moderate to severe ankylosing spondylitis (AS)
   AND
2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor

G. NON-RADIOGRAPHIC AXIAL SPONDYLOARTHRITIS (nr-axSpA):
1. Documented diagnosis of adult-onset axial spondylarthritis (nr-axSpA)
   AND
2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor

H. CROHN'S DISEASE (CD):
1. Documentation of a diagnosis of Crohn's Disease (CD)
   AND
2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor
   OR
3. If TNF-inhibitor is clinically inadvisable, documentation of treatment failure or serious side effects to a trial of a systemic therapy indicated for Crohn's Disease

I. POLYARTICULAR JUVENILE IDIOPATHIC ARTHRITIS (pJIA):
1. Documented diagnosis of polyarticular juvenile idiopathic arthritis (pJIA) in a pediatric member
   AND
2. Documentation of treatment failure or serious side effects to a trial (> 3 months) of one TNF-inhibitor

J. GIANT CELL ARTERITIS (GCA):
1. Documented diagnosis of Giant Cell Arteritis (GCA)

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CONTINUATION OF THERAPY:
A. FOR ALL INDICATIONS:
1. Prescriber attests to or clinical reviewer has found no evidence of intolerable adverse effects or drug toxicity (e.g., neutropenia, lymphopenia, anemia, etc.)
   AND
2. Prescriber attests to or clinical reviewer has found positive clinical response as demonstrated by low disease activity and/or improvements in the condition's signs and symptoms.
   AND
3. For ULCERATIVE COLITIS or CROHN'S DISEASE, **30 mg once daily**:
   Documentation of refractory, severe or extensive disease AND documentation 15 mg once daily was inadequate.
   AND
4. For ATOPIC DERMATITIS, **30 mg once daily**:
   Documentation 15 mg once daily was inadequate.

DURATION OF APPROVAL:
Initial authorization: 12 months, Continuation of Therapy: 12 months

PRESCRIBER REQUIREMENTS:
None

AGE RESTRICTIONS:
Rheumatoid Arthritis, Ulcerative Colitis, Ankylosing Spondylitis, Non-radiographic axial spondyloarthritis, Crohn's disease, Giant Cell Arteritis: 18 years of age and older
Atopic Dermatitis: 12 years of age and older
Psoriatic Arthritis: 2 years of age and older
Juvenile Idiopathic Arthritis: 2 years of age and older

QUANTITY: See specific quantity limit requirements below

Psoriatic Arthritis:
Adults: 15mg once daily
Pediatric Patients 2 to less than 18 years of age: based on body weight

Body Weight	Rinvoq LQ	Rinvoq tablets
10 kg to less than 20 kg	3 mg (3 mL oral solution) twice daily	Not recommended
20 kg to less than 30 kg	4 mg (4 mL oral solution) twice daily	Not recommended
30 kg and greater	6 mg (6 mL oral solution) twice daily	15 mg (one 15 mg tablet) once daily

NOTE: Rinvoq LQ oral solution is not substitutable with Rinvoq extended-release tablets. Changes between Rinvoq LQ oral solution and Rinvoq extended-release tablets should be made by the health care provider.

Ulcerative Colitis:
Induction dosage: 45 mg once daily for 8 weeks. THEN recommended maintenance dosage is 15 mg once daily.
- A maintenance dosage of **30 mg once daily** may be considered for patients with refractory, severe, or extensive disease. Discontinue Rinvoq if adequate therapeutic response is not achieved with the 30 mg dosage.

Atopic Dermatitis:
Adults 65 Years of Age and Older: Recommended dosage is 15 mg once daily.
Pediatric Patients 12 Years of Age and Older Weighing at Least 40 kg and Adults Less Than 65 Years

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of Age: Initiate treatment with 15 mg orally once daily.
- If an adequate response is not achieved, consider increasing the dosage to **30 mg orally once daily**.

Crohn's Disease:
Induction dosage: 45 mg once daily for 12 weeks. THEN recommended maintenance dosage is 15 mg once daily.
- A maintenance dosage of **30 mg once daily** may be considered for patients with refractory, severe, or extensive disease. Discontinue Rinvoq if adequate therapeutic response is not achieved with the 30 mg dosage.

Polyarticular Juvenile Idiopathic Arthritis:
The recommended dosage is based on body weight

Body Weight	Rinvoq LQ	Rinvoq tablets
10 kg to less than 20 kg	3 mg (3 mL oral solution) twice daily	Not recommended
20 kg to less than 30 kg	4 mg (4 mL oral solution) twice daily	Not recommended
30 kg and greater	6 mg (6 mL oral solution) twice daily	15 mg (one 15 mg tablet) once daily

NOTE: Rinvoq LQ oral solution is not substitutable with Rinvoq extended-release tablets. Changes between Rinvoq LQ oral solution and Rinvoq extended-release tablets should be made by the health care provider.

PLACE OF ADMINISTRATION:
The recommendation is that oral medications in this policy will be for pharmacy benefit coverage and patient self-administered.

DRUG INFORMATION

ROUTE OF ADMINISTRATION:
Oral

DRUG CLASS:
Antirheumatic-Janus Kinase (JAK) Inhibitors

FDA-APPROVED USES:
Indicated for the treatment of:
- Adults with moderately to severely active rheumatoid arthritis who have had an inadequate response or intolerance to one or more TNF blockers.
- Adults and pediatric patients 2 years of age and older with active psoriatic arthritis who have had an inadequate response or intolerance to one or more TNF blockers
- Adults and pediatric patients 12 years of age and older with refractory, moderate to severe atopic dermatitis whose disease is not adequately controlled with other systemic drug products, including biologics, or when use of those therapies are inadvisable.
- Adult patients with moderately to severely active ulcerative colitis who have had an inadequate response or intolerance to one or more TNF blockers. If TNF blockers are clinically inadvisable, patients should have received at least one approved systemic therapy prior to use of Rinvoq.
- Adults with active ankylosing spondylitis who have had an inadequate response or intolerance to one or more TNF blockers
- Adults with active non-radiographic axial spondyloarthritis with objective signs of inflammation who have had an inadequate response or intolerance to TNF blocker therapy
- Adult patients with moderately to severely active Crohn's disease who have had an inadequate response or intolerance to one or more TNF blockers. If TNF blockers are clinically inadvisable, patients should have received at least one approved systemic therapy prior to use of Rinvoq.
- Patients 2 years of age and older with active polyarticular juvenile idiopathic arthritis who have had

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an inadequate response or intolerance to one or more TNF blockers
- Adults with giant cell arteritis

Limitations of Use: Rinvoq/Rinvoq LQ is not recommended for use in combination with other JAK inhibitors, biologic DMARDs, biologic therapies for ulcerative colitis, biologic therapies for Crohn's disease, or with potent immunosuppressants such as azathioprine and cyclosporine.

COMPENDIAL APPROVED OFF-LABELED USES:
None

APPENDIX

APPENDIX:

Illinois (Source: Illinois General Assembly)
"(215 ILCS 134/45.1) Sec. 45.1. Medical exceptions procedures required. (c) An off-formulary exception request shall not be denied if: (1) the formulary prescription drug is contraindicated; (2) the patient has tried the formulary prescription drug while under the patient's current or previous health insurance or health benefit plan and the prescribing provider submits evidence of failure or intolerance; or (3) the patient is stable on a prescription drug selected by his or her health care provider for the medical condition under consideration while on a current or previous health insurance or health benefit plan. (d) Upon the granting of an exception request, the insurer, health plan, utilization review organization, or other entity shall authorize the coverage for the drug prescribed by the enrollee's treating health care provider, to the extent the prescribed drug is a covered drug under the policy or contract up to the quantity covered. (e) Any approval of a medical exception request made pursuant to this Section shall be honored for 12 months following the date of the approval or until renewal of the plan."

BACKGROUND AND OTHER CONSIDERATIONS

BACKGROUND:
Rinvoq (Upadacitinib) is a Janus kinase (JAK) inhibitor. JAKs are intracellular enzymes which transmit signals arising from cytokine or growth factor-receptor interactions on the cellular membrane to influence cellular processes of hematopoiesis and immune cell function. Within the signaling pathway, JAKs phosphorylate and activate Signal Transducers and Activators of Transcription (STATs) which modulate intracellular activity including gene expression. Rinvoq modulates the signaling pathway at the point of JAKs, preventing the phosphorylation and activation of STATs.

Exclusions/Discontinuation:
Based on findings in animal studies, Rinvoq/Rinvoq LQ may cause fetal harm when administered to a pregnant woman. Verify the pregnancy status of patients of reproductive potential prior to starting treatment. Advise females of reproductive potential of the potential risk to the fetus and to use effective contraception during treatment with Rinvoq/Rinvoq LQ and for 4 weeks following completion of therapy.

OTHER SPECIAL CONSIDERATIONS:
Rinvoq (upadacitinib) has a Black Box Warning for serious infections, mortality, malignancy, major adverse cardiovascular events (MACE), and thrombosis.
Patients treated with Rinvoq/Rinvoq LQ are at increased risk for developing serious infections that may lead to hospitalization or death. Most patients who developed these infections were taking concomitant immunosuppressants such as methotrexate or corticosteroids. If a serious infection develops, interrupt Rinvoq/Rinvoq LQ until the infection is controlled.
Reported infections include:
• Active tuberculosis, which may present with pulmonary or extrapulmonary disease. Patients should be tested for latent tuberculosis before Rinvoq/Rinvoq LQ use and during therapy. Treatment for latent infection should be considered prior to Rinvoq/Rinvoq LQ use.
• Invasive fungal infections, including cryptococcosis and pneumocystosis.
• Bacterial, viral, including herpes zoster, and other infections due to opportunistic pathogens.

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The risks and benefits of treatment with Rinvoq/Rinvoq LQ should be carefully considered prior to initiating therapy in patients with chronic or recurrent infection. Patients should be closely monitored for the development of signs and symptoms of infection during and after treatment with Rinvoq/Rinvoq LQ, including the possible development of tuberculosis in patients who tested negative for latent tuberculosis infection prior to initiating therapy.
In a large, randomized, postmarketing safety study in rheumatoid arthritis (RA) patients 50 years of age and older with at least one cardiovascular risk factor comparing another Janus kinase (JAK) inhibitor to tumor necrosis factor (TNF) blockers, a higher rate of all-cause mortality, including sudden cardiovascular death, was observed with the JAK inhibitor.
Lymphoma and other malignancies have been observed in patients treated with Rinvoq. In RA patients treated with another JAK inhibitor, a higher rate of malignancies (excluding non-melanoma skin cancer (NMSC)) was observed when compared with TNF blockers. Patients who are current or past smokers are at additional increased risk.
In RA patients 50 years of age and older with at least one cardiovascular risk factor treated with another JAK inhibitor, a higher rate of major adverse cardiovascular events (MACE) (defined as cardiovascular death, myocardial infarction, and stroke), was observed when compared with TNF blockers. Patients who are current or past smokers are at additional increased risk. Discontinue Rinvoq/Rinvoq LQ in patients that have experienced a myocardial infarction or stroke.
Thromboses, including deep venous thrombosis, pulmonary embolism, and arterial thrombosis, have occurred in patients treated for inflammatory conditions with JAK inhibitors, including RINVOQ. Many of these adverse events were serious and some resulted in death. In RA patients 50 years of age and older with at least one cardiovascular risk factor treated with another JAK inhibitor, a higher rate of thrombosis was observed when compared with TNF blockers. Avoid Rinvoq/Rinvoq LQ in patients at risk. Patients with symptoms of thrombosis should discontinue Rinvoq/Rinvoq LQ and be promptly evaluated.

AVAILABLE DOSAGE FORMS

AVAILABLE DOSAGE FORMS:
Rinvoq TB24 15MG, 30MG, 45MG
Rinvoq LQ SOL 1MG/ML

REFERENCES

1. Illinois HFS Drugs with Stipulated PA Language per Contract for MCOs April 1, 2026.
2. Illinois Medicaid Preferred Drug List, Effective April 1, 2026.
3. Rinvoq (upadacitinib) extended-release tablets, for oral use; Rinvoq LQ (upadacitinib) oral solution [prescribing information]. North Chicago, IL: AbbVie; October 2025.

SUMMARY OF REVIEW/REVISIONS	DATE
CRITERIA CREATION	04/2026

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